HEALTH AND NUTRITION

Public Health Expenditure	0.8% (as % of GDP) FY2025	
Health Sector PSDP Allocation	19.375 (Rs. billion)	
Hospitals	1,934 (2025) P	
Basic Health Units	5,746 (2025) P	
Infant Mortality Rate	47 (Per 1,000 Births) (2024-25 PSLM)	
Life Expectancy	67.8 (2024)	
Registered Doctors	336,582 (2025) P	

Chapter 11

Health and Nutrition

Health underpins national development by strengthening human capability and enabling individuals and communities to thrive. Investing in health is crucial for realizing the potential of individuals and developing human capital in Pakistan. The country aims to align with the 2030 Agenda for Sustainable Development, particularly focusing on ensuring universal access to quality healthcare services without financial hardship. Over the years, Pakistan has made progress in expanding healthcare services, controlling infectious diseases, and improving key health indicators. However, the sector continues to face challenges, such as limited financial resources, unequal access to healthcare facilities, rapid population growth, and a high burden of both communicable and non-communicable diseases.

This chapter provides an overview of the health and nutrition sector in Pakistan, highlighting its structure, key challenges, recent developments and policy measures aimed at improving the overall health status of the population.

11.1 Health Profile of Pakistan

Pakistan's healthcare system comprises public and private sectors, with the government being the primary provider of preventive and basic healthcare services, while the private sector contributes significantly to curative care. Various national health programmes have been launched to address major health issues such as maternal and child health, immunization, nutrition, and disease control. Pakistan has made significant progress in key health indicators, reflecting sustained improvements in healthcare delivery and public health outcomes.

During the period under review, Pakistan recorded steady gains across major health indicators. Life expectancy at birth increased from 66.5 years in 2018 to 67.8 years in 2024, while the prevalence of HIV remained stable at 0.2 percent. The incidence of tuberculosis remained broadly stable, whereas maternal health improved considerably. As per PDHS/PMMS(2019), maternal health also improved, with the maternal mortality ratio declining from 276 to 186 per 100,000 live births

As per Pakistan Social Living Standard Measurement Surveys (PSLM), notable progress is also observed in child health, as immunization coverage rose from 68 percent in 2018–19 to 73 percent in 2024–25, alongside reductions in neonatal mortality from 41 to 35 per 1,000 live births and infant mortality from 60 to 47 per 1,000 live births. Overall, these improvements highlight meaningful advancements in maternal and child healthcare and strengthened public health services in Pakistan.

Table 11.1 offers a comprehensive overview of various metrics of Pakistan's health sector.

Table 11.1: Health Indicators of Pakistan

Indicators	2018	2024
Life expectancy at birth (years)	66.5	67.8
Prevalence of HIV, Total (% of population ages 15-49)	0.2	0.2
Incidence of Tuberculosis (per 100,000 people)	265	266
Maternal mortality ratio (per 100,000 births) as per PDSH/PMMS	276 (2007)	186 (2019)

Table 11.1: Health Indicators of Pakistan

	2018-19	2024-25
Immunization, (% of children immunized)	68	73
Neonatal Mortality rate (per 1000 live births)	41.0	35.0
Mortality rate, Infant (per 1000 live births)	60	47

Source: World Development Indicators, PSLM 2024-25

Regional comparison

In the South Asian region, the health landscape remains challenging, although there has been slight improvement in recent years. As of 2024, the average life expectancy of South Asian countries stood at 72.6 years. Moreover, maternal mortality rates recorded at 120 in 2023, as compared to 155 per 100,000 live births in Pakistan (Table 11.2). The infant mortality rate in South Asia was 23.2 per 1,000 live births in 2024 while 47 per 1000 live births in Pakistan. Nevertheless, Pakistan is making efforts to improve its health sector outcomes and bridge the gap with other South Asian countries. The key health sector indicators for South Asia are summarized in the following table to facilitate a comparison with Pakistan's performance.

Table 11.2: Regional Comparison of Health Indicators of South Asia and Pakistan (2024)

Indicators	South Asia	Pakistan
Life expectancy at birth (years)	72.6	67.8
Maternal mortality ratio (per 100,000 births) * 2023	120	155
Birth rate (Crude) per 1000 people	16.4	27.4
Mortality rate, Infant (per 1000 live births)	23.2	47.0
Prevalence of HIV (total %age of population 15-49 years of age)	0.19	0.20

* South Asia's data as per modeled estimates by WDI

Source: World Development Indicators (WDI), World Health Organization (WHO), PSLM 2024-25

Pakistan's health indicators have shown modest improvement compared to previous years. However, when compared to the South Asian region, Pakistan still has scope for further improvement in key development indicator.

Pakistan's Progress on SDGs

Pakistan has made steady progress toward achieving SDG 3 — Good Health and Well-Being — despite facing challenges. As per the Sustainable Development Report 2025, Pakistan ranked at 140 out of 167 countries. The maternal mortality and under 5 mortality has shown moderate improvement, reflecting positive developments in maternal health services. Additionally, Pakistan continues its efforts to reduce neonatal mortality and improvement in Universal Health Coverage, still require accelerated action. The country has made advances in expanding immunization coverage. These achievements underscore Pakistan's commitment to improving health outcomes and building a resilient healthcare system. Furthermore, investment and steps are in progress to fully meet SDG 3 targets.

Table 11.3: Progress of SDG-3 indicators

Sr. #	SDG-3 Indicators	SDG 2024	SDG 2025
01	Surviving infants who received 2 WHO-recommended vaccines (%)	82.0	84.0
02	Universal health coverage (UHC) index of service coverage (0 = worst, 100 = best.)	45.0	45.2
03	Age-standardized death rate due to cardiovascular disease, cancer, diabetes, or chronic respiratory disease in adults aged 30 to 70 years (%)	29.4	25.5

Source: Sustainable Development Report 2024 and 2025

11.1-a Human Resource in the Healthcare Sector

To provide healthcare services to the population, it is important to have sufficient healthcare professionals. Therefore, increasing the number of healthcare workers is essential to ensure the efficient delivery of health services. The table below shows the number of healthcare personnel in the country. In 2025, there were 336,582 registered doctors and 42,118 registered dentists, compared to 319,572 doctors and 39,088 dentists in 2024 (Table 11.4). This represents an increase in the number of registered doctors and dentists by 5.3 percent and 7.8 percent, respectively.

Table 11.4: Registered Medical and Paramedical personnel

Health manpower	2019	2020	2021	2022	2023	2024	2025(P)
Doctors	233,261	245,987	266,430	282,383	299,113	319,572	336,582
Dentists	24,930	27,360	30,501	33,156	36,032	39,088	42,118
Nurses	112,123	116,659	121,245	127,855	134,708	138,391	138,391
Midwives	41,810	43,129	44,693	46,110	46,404	46,801	46,801
Lady health workers	20,565	21,361	22,408	24,022	26,405	29,163	29,163

P: Provisional, Note: Data is reported on a calendar year basis

Source: Pakistan Bureau of Statistics

Box-I: Champion Country at the Global Caregiver Forum

The first-ever Global Caregiver Forum brought together senior representatives from governments, UNICEF, WHO and international advocacy organizations to share best practices and policy perspectives on universal caregiver support as essential social infrastructure. The event focused on developing a comprehensive global roadmap for scaling up evidence-based parenting programmes aimed at supporting caregivers, strengthening families, and safeguarding children's rights and development.

Pakistan was recognized as a Champion Country for its pioneering work on Early Childhood Development at the Global Caregiver Forum. The recognition also reflects ongoing efforts to integrate caregiving into national health and social protection frameworks, ensuring more inclusive and sustainable healthcare delivery. The country presented evidence-based, people-centered policies that place children, families, and human capital at the core of national development, reflecting Pakistan's growing role in shaping the global discourse on early childhood investment and human capital development.

Source: Ministry of Planning, Development and Special Initiative

11.1-b Health expenditures

Public health expenditures play a crucial role in achieving universal health coverage. In FY 2025, health expenditure increased by 1.9 percent, reaching Rs 942.2 billion compared to Rs 924.9 billion in FY 2024. In Pakistan, health expenditure as a percentage of GDP is currently low, though the allocation is anticipated to gradually increase in the years ahead. Table 11.5 below shows the consolidated federal and provincial health expenditure position over the last 6 years, while Figure 11.1 illustrates Pakistan's total health expenditures.

Fig-11.1: Expenditures on Health Sector (Rs million)

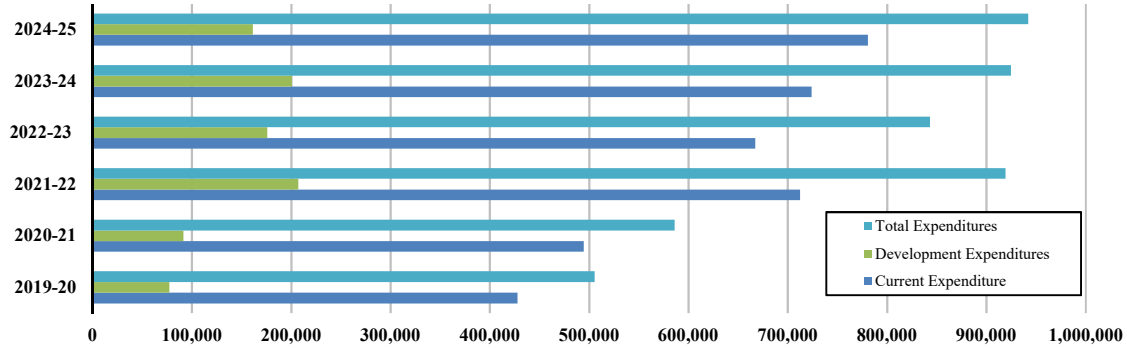


Table 11.5: Federal and Provincial Health Expenditures Rs million

Years		Current Expenditure	Development Expenditures	Total Expenditures	Percent of GDP
2019-20	Federal	11,439	12,856	24,295	1.1
	Punjab	220,854	40,403	261,257	
	Sindh	115,303	3,815	119,118	
	Khyber Pakhtunkhwa	58,289	15,132	73,421	
	Balochistan	22,030	5,290	27,320	
	Pakistan	427,915	77,496	505,411	
2020-21	Federal	41,309	9,613	50,922	1.0
	Punjab	221,469	52,705	274,174	
	Sindh	150,668	4,057	154,725	
	Khyber Pakhtunkhwa	56,179	20,778	76,957	
	Balochistan	24,981	4,511	29,492	
	Pakistan	494,606	91,664	586,270	
2021-22	Federal	153,030	9,530	162,560	1.4
	Punjab	258,860	152,367	411,227	
	Sindh	177,735	10,047	187,782	
	Khyber Pakhtunkhwa	95,302	28,865	124,167	
	Balochistan	27,362	6,320	33,682	
	Pakistan	712,289	207,129	919,418	
2022-23 (P)	Federal	31,397	4,495	35,892	1.0
	Punjab	303,056	147,554	450,610	
	Sindh	199,474	5,158	204,632	
	Khyber Pakhtunkhwa	111,368	10,980	122,348	
	Balochistan	22,012	7,685	29,697	
	Pakistan	667,307	175,872	843,179	
2023-24	Federal	45,007	8,147	53,154	0.9
	Punjab	377,282	174,595	551,876	
	Sindh	130,562	3,603	134,165	
	Khyber Pakhtunkhwa	129,028	14,468	143,496	
	Balochistan	42,194	-	42,194	
	Pakistan	724,072	200,813	924,885	
2024-25	Federal	53,966	21,288	75,254	0.8
	Punjab	271,293	92,185	363,478	
	Sindh	245,305	7,607	252,912	
	Khyber Pakhtunkhwa	152,364	22,682	175,046	
	Balochistan	57,777	17,721	75,498	
	Pakistan	780,704	161,484	942,188	

P: Provisional

Source: PRSP Budgetary Expenditures, Finance Division

Box-II: Draft National Health and Population Policy (2026–2035)

Following the completion of the National Health Vision 2016–25, the Inter-Ministerial Health and Population Council in May 2024 approved the initiation of a new National Health and Population Policy for the next decade. The policy provides a unified national framework to guide health and population actions at federal and provincial levels. It reaffirms Pakistan's commitment to the Sustainable Development Goals (SDGs), with Universal Health Coverage (UHC) as a central pillar. The policy also incorporates emerging priorities such as food security, climate change impacts, demographic transition, and multi-sectoral determinants of health. Alignment with national development priorities is ensured through integration with the 5Es Framework and other strategic planning instruments.

Key Policy Priorities:

1. Good governance for better health outlook along with fair and sustainable financing

2. Universal access to health services and addressing emerging challenges

3. Reducing high population growth

4. Tackling malnutrition and strengthening health security with competent allied and health workforce

5. Ease of access to medication, advancing digitization, dialogue, and evidence-based decision-making

Source: Ministry of National Health Services, Regulation and Coordination

11.1-c Health Sector Projects of PSDP

Under the PSDP 2025–26, health-related development interventions are mainly reflected under the Ministry of National Health Services, Regulations and Coordination (M/o NHSR&C), with selected health projects also appearing under Provinces and Special Areas, Interior Division, and Defence Division. The total approved cost of these health-related projects stands at Rs 363,544.4 million. Expenditure up to 30-06-2025 amounted to Rs 45,480.5 million, leaving a throw-forward of Rs 318,063.8 million as of 01-07-2025.

For FY 2026, an allocation of Rs 19,375.8 million has been made for these projects. Of the total, NHSR&C accounts for the major share with Rs 14,053.5 million, followed by Provinces and Special Areas with Rs 4,413.7 million, Defence Division with Rs 808.6 million, and Interior Division with Rs 100.0 million. This reflects continued federal support for hospital infrastructure, specialized care, disease control, and medical institutions across the country.

Table 11.6: Health Sector Projects in the PSDP for FY 2025 Rs million

Sr. No.	Name of Ministry /Organisation	No. of Projects	Total Cost	2025-26 PSDP Allocation
1	M/o NHSR&C	23	311,669.2	14,053.5
2	Provinces and Special Areas	10	39,195.2	4,413.7
3	Interior Division	1	6,479.9	100.0
4	Defence Division	3	6,200.0	808.6
	Grand Total	**37**	**363,544.4**	**19,375.8**

Source: Ministry of Planning, Development & Special Initiatives. (M/o PD&SI)

11.1-d Key Health Sector Initiatives

The Government has undertaken significant steps to strengthen the health sector as a cornerstone of national development by enhancing human capital and enabling communities to thrive. The federal and provincial governments have undertaken the following initiatives and interventions.

i) Development of the National Digital Health Hub

The Health Information System (HIS) in Pakistan is critical for enabling data-driven decision-making and improving health outcomes. HIS supports evidence-based policymaking, equitable resource allocation, and health system monitoring, align with the upcoming National Health and Population Policy (2026–35) and the goals of Universal Health Coverage (UHC) and SDGs. Key national platforms, such as the Pakistan Health Information System (PHIS) Dashboard and the Pakistan Health Knowledge Hub (PHKH), aim to consolidate data and promote evidence-based decision-making. However, challenges around data integration, quality, storage, and stakeholder engagement limit their effectiveness. A unified, consolidated HIS that integrates administrative, clinical, epidemiological, and surveillance data is essential to provide a holistic view of the health sector.

Central to this vision is the development of a national dashboard that offers real-time insights into health system performance through advanced analytics and AI-powered visualization tools. In this context, the M/oNHSR&C is developing the National Digital

Health Hub outlining a transformational architecture to unify Pakistan's fragmented health information ecosystem into an interoperable, analytics-driven platform. The focus is to build the system architecture and operational software for a national dashboard that consolidates diverse health data streams, provides actionable analytics to stakeholders, and establishes a comprehensive knowledge repository for timely access to policies and documents.

ii) Pakistan Multisectoral Health Workforce Strategy (2025–34)

The Pakistan Multisectoral Health Workforce Strategy (PMHWS) 2025–34, developed by the M/o NHSR&C with broad stakeholder engagement across federal, provincial, and allied sectors, represents a landmark framework for strengthening Pakistan's health workforce in support of health security, and emergency preparedness. This strategy adopts a One Health approach, integrating human, animal, and environmental health competencies, to ensure that the health workforce is resilient, skilled, and capable of effectively responding to current and future public health challenges, including pandemics, zoonotic threats, and climate-linked health risks. By aligning with international frameworks such as the International Health Regulations (IHR 2005) and the National Action Plan for Health Security 2022–28, and incorporating surge capacity training and multisectoral coordination mechanisms, the strategy seeks to foster cross-sectoral collaboration, improve workforce distribution and performance, and ultimately enhance the nation's capacity to deliver equitable, quality health services for all Pakistanis.

iii) Pakistan Multisectoral Surge Capacity Building Package (PMSCBP)

In 2023, Joint External Evaluation (JEE 2023) assessed Pakistan's surge capacity under IHR Indicator D3.4 at a low level, highlighting the absence of institutionalized, multisectoral approaches to surge preparedness, readiness, and deployment. While, multiple institutions contribute to emergency response, surge arrangements have largely remained fragmented, ad hoc, and unevenly applied across sectors and levels.

In this regard, the PMSCBP is a national, multisectoral surge capacity building framework designed to strengthen how institutions scale people, resources, coordination mechanisms, and systems during public health emergencies and other high impact events. The PMSCBP adopts a One Health and all-hazards approach and is intended for application across human health, animal health, environmental health, food safety, emergency response, and information systems. It supports preparedness, readiness, response, and validation as connected system functions rather than isolated activities.

Pakistan continues to experience recurrent public health emergencies and other high-impact events, including infectious disease outbreaks, climate-induced disasters, zoonotic threats, food safety incidents, and environmental health events. These emergencies have repeatedly demonstrated that surge capacity is not only a workforce issue, but a system function that depends on how institutions organise, coordinate, activate, and scale under pressure.

iv) Pakistan Nursing and Midwifery Policy Framework (2025–34)

The Pakistan Nursing and Midwifery Policy Framework is a strategic document, developed to strengthen and transform the nursing and midwifery workforce as a cornerstone of the country's health system. The Framework responds to longstanding challenges in governance, education, workforce distribution, and quality assurance, aiming to enhance regulatory oversight, expand education and training capacity, improve labour market dynamics, and strengthen standards of professional practice across Pakistan. The Framework supports the development of a competent, resilient, and well-regulated nursing and midwifery workforce that is better equipped to deliver equitable, high-quality care and contribute to UHC. Additionally, it outlines initiatives to boost nursing education infrastructure, facilitate international recognition and employment opportunities, and align Pakistan's nursing standards with global best

practices, thereby reinforcing the health system's capacity to meet current and future health challenges.

v) National Anaemia Reduction Initiatives

Addressing micronutrient deficiencies remains a priority for improving maternal, child, and adolescent health outcomes. In this context, the Government finalized the National Anaemia Reduction Plan (2025–2030), adopting a life-course approach targeting adolescent girls, women of reproductive age, pregnant and lactating women, and young children. The plan emphasizes prevention, screening, treatment, and multisectoral coordination, integrating nutrition-sensitive and nutrition-specific interventions within primary healthcare and community platforms. Implementation of this framework is expected to contribute to improved health outcomes and enhanced labour productivity over the medium to long term.

vi) National Prime Minister's Programme for Prevention and Control of Diabetes Mellitus

In view of the rising burden of non-communicable diseases, particularly diabetes mellitus, the Government approved the National Prime Minister's Programme for Prevention and Control of Diabetes Mellitus (2024–2029) as a five-year national initiative beginning with Islamabad Capital Territory, Azad Jammu & Kashmir, and Gilgit-Baltistan, and subsequently expanding to all provinces with total capital outlay of Rs 6.8 bn.

Key actions undertaken during the review period include establishment of project governance structures, operationalization of the Project Management Unit, initiation of procurement processes for diagnostics and medicines, capacity building of healthcare staff, and development of monitoring and surveillance mechanisms, including a national diabetes registry.

During the year, a nationwide awareness drive was advanced, highlighted by the observance of World Diabetes Day (10–14 November 2025), comprising screening camps, public awareness activities, media outreach, and stakeholder engagement across federal health institutions and communities. The programme aims to strengthen early detection and management of diabetes, reduce long-term complications, and mitigate productivity losses associated with non-communicable diseases, thereby contributing to improved population health and human capital outcomes.

vii) Prime Minister's Programme for Elimination of Hepatitis-C

To address the high burden of Hepatitis C infection, the Government approved the Prime Minister's Programme for the Elimination of Hepatitis C Infection (2024–2027). With an estimated prevalence of 7.5 percent among the population aged 12 years and above, the programme aims to screen, test and treat 50 percent of the eligible population through a standardized national approach.

During the review period, key milestones included authorization from the Planning Division, operationalization of programme bank accounts, notification of the Steering Committee, and partial staffing of the Project Management Unit, while pilot screening and treatment were completed in selected districts of Sindh and Balochistan, with expansion initiated in Azad Jammu & Kashmir and planned in Khyber Pakhtunkhwa. Procurement of rapid diagnostic tests, PCR kits, and antiviral medicines was advanced in accordance with Public Procurement Regulatory Authority rules, alongside coordination with national institutions for data verification, digital tracking, and logistics. The programme adopts a comprehensive service delivery model providing free screening, confirmatory testing, and treatment, supported by national awareness campaigns and strengthened provincial health systems, and is expected to significantly reduce disease burden, prevent complications such as liver cirrhosis and cancer, lower future healthcare costs, and enhance productivity and human capital outcomes.

viii) Cancer treatment

Pakistan Atomic Energy Commission (PAEC) has made significant contribution in

management, prevention and control of cancer and other non-communicable diseases. It is serving the humanity suffering from cancer sine 1960 by establishing first nuclear facility in Karachi and currently twenty one atomic energy cancer hospitals are operational in all over Pakistan.

About 80 percent of Pakistan's cancer burden is catered to by PAEC hospitals, as cancer patients are treated in multiple cancer facilities of Atomic Energy Cancer Hospitals (AECHs) irrespective of their stage and financial status. Over 1.0 million procedures are carried out annually in radiotherapy, radiology and nuclear medicine departments and over 2.0 million procedures are carried out in pathology department of AECHs. At present, there are 2300 personnel are employed in hospitals, comprising 250 doctors, 110 scientists and radio pharmacists, 30 engineers and 160 nurses.

Moreover, during July-March FY 2026, the radiopharmaceuticals production involved 293 hours operation of research reactors with the irradiation of 344 target materials. It includes 261 consignments for throid cancer, 11,484 vials of cold kits and adenosine injections etc.

The following targets have been achieved by FY 2026:

- Kashmir Institute of Nuclear Medicine, Oncology and Radiotherapy (KINOR) Muzaffarabad, the 21st Atomic Energy Cancer Hospital has been inaugurated and start functioning. Moreover, State of the art LINAC equipment is reached at KINOR and installation is in progress.
- Students/ fellows attended AECHs for training in nuclear medicine, radiation therapy, oncology, and radiology. AECHs are providing teaching and training facilities to students of Ph.D., FCPS, MCPS, MS, MSc, BS etc. More than 1000 students got training last year in disciplines mentioned.
- New state of the art CT-Scanner for diagnostic radiology and modern True Beam LINAC reached at AECH INMOL in said period and the installation is under process. For strengthening Nuclear Medicine department and to meet patient load one more Gamma camera was installed and commissioning of the equipment is under process.
- AECH BINO has installed MRI for diagnostic radiology and True Beam Linear Accelerator for radiation therapy. AECH PINUM installed Gamma Camera and diagnostic X-ray.
- AECH CENAR has installed state of the art Chemistry and special chemistry analyzer (Roche C111 & E411) for upgradation of Pathology lab.
- Research projects of IAEA TC / RCA projects and collaboration with different national and international organizations is continued. Approximately 30–40 national and international researches by AECHs were published in 6 months, along with multiple national and international projects
- Multiple events about cancer awareness were organized in said period in all AECHs, especially the whole month of October was celebrated for breast cancer awareness. More than 225 lectures, presentations, awareness talks, conferences, meetings and medical camps were organized in said period.

ix) Capacity building and Specialized Trainings/Initiatives for Health Sector preparedness and response during disasters

The National Disaster Management Authority (NDMA) took various measures to avoid humanitarian crises during disasters. The following are capacity-building and training programmes conducted to respond to the health issues immediately during disasters

- To improve the datasets for disaster management, on 18th July, 2025 a baseline data validation workshop for the Multi-Hazard Vulnerability & Risk Assessment (MHVRA) of District Nushki was held at BUITEMS. The session focused on reviewing collected data for accuracy and completeness. Moving forward, NDMA developed a simplified baseline data collection form to assist PDMA and district officials standardizing data gathering.

- Chemical industrial disasters pose serious risks to public health and require coordinated operational, technical, and medical responses. NDMA, as Pakistan's leading disaster management authority, regularly conducts simulation exercises, train responders, improve coordination, and identify gaps. In this context, a joint exercise was conducted at FFCL Plant, Mirpur Mathelo (12–13 February 2026), involving key stakeholders, combining community awareness with a simulation of ammonia leakage and fire emergency to evaluate response and coordination mechanisms.
- NDMA strengthened gender-responsive and inclusive disaster preparedness through key national initiatives in health, education, social protection, and climate change. These included Minimum Initial Service Package (MISP) and Gender-Based Violence in Emergencies (GBViE) trainings, school safety, audits for teachers, RFID-based support for vulnerable groups, inclusive early warning systems, and policy reforms covering transgender persons and persons with disabilities. Whereas 2,645 girls guide were trained under Disaster Risk Reduction (DRR) badge initiative till March 2026.
- NDMA Signed a Letter of Agreement (LOA) with United Nation University Institute for Water, Environment and Health, Canada to advance academic research and policy dialogue on disaster management, climate change and regional cooperation.

11.1-e Provincial Performance in the Health Sector in FY 2026

Punjab

During 2025–26, the Government of Punjab launched a comprehensive healthcare reform programme under the Punjab Primary and Secondary Healthcare Services Act, 2025, to strengthen community, primary, and secondary healthcare services across the province. More than 50,000 health workers were deployed under the Community Health Inspectors (CHI) initiative, with over 45 million premises and 8.29 million families digitally mapped for door-to-door healthcare outreach. The government outsourced 2,506 Maryam Nawaz Health Clinics (MNHCs) to improve primary healthcare access, while 115 Maryam Nawaz Hospitals (MNHs) were operationalized to revitalize rural secondary healthcare facilities. Digital reforms included EMR systems, HMIS integration, CCTV-based monitoring, and FlexiCare attendance and wage systems to improve transparency and accountability. The Directorate of Monitoring and Evaluation conducted 29,499 facility visits between September 2025 to March 2026 to ensure service quality. At the district level, four Cath Labs became functional by March 2026 and performed 1,135 cardiac procedures, including 756 angiographies and 379 angioplasties. Human resource reforms generated over 50,000 employment opportunities, including 30,000 jobs under the CHI programme and more than 10,000 positions in MNHCs. The government also initiated Schools of Nursing and Midwifery across major districts, with the first batch enrolling 308 students in BS Nursing and BS Midwifery programmes. Overall, these initiatives represent a shift toward a digitally enabled, performance-based, and integrated healthcare system aimed at improving healthcare access, quality, and progress toward UHC.

Sindh

The Health Department, Sindh, undertook key policies and administrative measures to consolidate ongoing initiatives and improve service delivery. During FY 2026 key priorities of Health Department Sindh are ensuring Universal Health Coverage through provision of equity-based health services throughout the province. The following key steps taken in FY 2026 to address uneven distribution of healthcare services. In medical education, Provincial Quality Control Board (PQCB) was constituted and the new Medical & Dental college Admission Test (MDCAT) Policy has been approved by Chief Minister. Moreover, policy on Bachelors in Science of Nursing (BSN) Degree Programme is notified along with evening shift policy for induction of 1150 students in BSN Degree Programme in Public Sector Colleges is notified and is implemented.

Furthermore, Local Government and Rural Development Department Sindh has signed an

MOU with NADRA for registration of birth and death in medical facilities, with ambassador of UAE for taking over the possession of 20 Bedded hospital at Shahdadpur, and HELP Organization for providing quality health services in 09 Government Dispensaries of Tharparkar. The last but not least, Regulations on Safe Blood Transfusion in Sindh have been notified after approval of provincial cabinet of Sindh.

Khyber Pakhtunkhwa

In Khyber Pakhtunkhwa, health is presented as a single integrated sector in the ADP 2025–26. The sectoral composition covers basic health, general hospitals, medical education and training, preventive programmes, social action programme, and teaching hospitals, indicating a broad development focus across different levels of care and service delivery.

The health sector in Khyber Pakhtunkhwa comprises 183 schemes with a total cost of Rs 258,183 million. The FY 2026 allocation stands at Rs 46,819.0 million, including local and foreign-aided components, while expenditure up to June 2025 is reported at Rs 71,869.3 million. This indicates that the province is carrying a sizeable ongoing health portfolio into FY 2026 while continuing to allocate substantial resources to health sector expansion and strengthening.

Balochistan

Balochistan's PSDP 2025–26 identifies health as one of the province's major social sectors. The department-wise abstract shows health as a significant component of the provincial development portfolio.

Balochistan's Health Department comprises 242 schemes with a total cost of Rs 110,523.2 million. The allocation for FY 2026 stands at Rs 16,402.7 million, while expenditure up to June 2025 is Rs 33,417.1 million. These figures combine Government of Balochistan and foreign project assistance components, and reflect continued investment in provincial health service delivery and related infrastructure under the PSDP framework.

Gilgit Baltistan

During FY 2026, the Government of Gilgit-Baltistan undertook several key measures to strengthen health infrastructure through major PSDP initiatives, including the establishment of a 250-bedded hospital at Skardu, 50-bedded Cardiac Hospital Phase-I at Gilgit, Medical and Nursing College at Gilgit, up-gradation of the Provincial Headquarters Hospital, and establishment of a 300-bedded Mother and Child/ICT Hospital at Chilas. Significant implementation progress was achieved under these initiatives, including completion of feasibility studies, topographical surveys, geotechnical investigations, Environmental Impact Assessment (EIA), Initial Environmental Examination (IEE), detailed design, project estimation, and master planning. The above measures demonstrates continued institutional progress and commitment to strengthening tertiary healthcare and medical education facilities in the region.

Azad Jammu and Kashmir

The Government of Azad Jammu and Kashmir (AJK) is fully committed to quality healthcare facilities for the general public. The strategies adopted for the health sector are to interpret the government's priority agenda to achieve universal health coverage by 2030.

Health coverage in AJK is still inadequate. There are approximately 2950 Hospital beds available in the area averaging one bed per 1558 people. The total number of doctors is 1445 out of which there are 71 Health Managers, 436 specialists, 93 Dental Surgeons and 845 Medical Officers. There is a ratio of 0.95 Per 10000 Population in respect of specialist, 1.83 in respect Medical Officers of and 0.20 in respect of Dentists Surgeon.

AJK Government has taken multiple initiatives to improve the health situation. Sehat Sahulat Programme (SSP) is a flagship programme of social health protection and a landmark towards

attaining UHC. The total 1.25 million families of AJK were entitled for indoor treatment facilities from public and private sector hospitals all over the country. Though programme was discontinued since 31st December, 2023 but has been restored in AJK since 16th January 2026, through the support of Federal government of Pakistan

Moreover, Family Planning and Primary Healthcare Programme (FP&PHC), has recruited more than 3,068 LHWs in AJK covering 67 percent of population with services encompassing the health conditions of women and children through improved sanitation, birth spacing, iron supplementation, more extensive vaccination coverage, and through Ante-Natal Coverage (ANC) and Post-Natal Coverage (PNC).

During December 2025, approximately 0.569 million children, were vaccinated through the National Immunization Day (NID) campaign across the AJK. Vaccine procurement and social mobilization are undertaken by UNICEF, with WHO collaborations on operational activities, monitoring and environmental surveillance. Moreover, the State of AJK is Polio Free from last Twenty years.

In addition to that, National TB Control Programme (NTP) has achieved over 85 present DOTS coverage in the public sector.

11.2 Nutrition Profile of Pakistan

Nutrition is fundamental to human development, economic productivity, and the achievement of sustainable development outcomes. The nutritional status of the population, particularly children in early childhood, adolescents, and women of reproductive age, shapes health trajectories, educational attainment, and workforce participation. A well-nourished population supports higher productivity, reduces the burden on public health systems, and contributes to sustained and inclusive economic growth. Pakistan faces a significant burden of malnutrition in multiple forms.

Recent evidence from the Household Integrated Economic Survey (HIES) 2024-25 indicates increasing constraints in households' ability to access adequate and quality diets (Table 11.7). Household-level food consumption data from HIES further illustrates the changing dietary patterns underlying these trends. Monthly per capita consumption of key food groups between 2018-19 and 2024-25 reveals a decline in staple cereals, pulses, milk, and meat, the primary sources of protein, while consumption of vegetable ghee has risen markedly. These shifts suggest that rising food prices and constrained household incomes are compressing dietary diversity, with lower-income households substituting cheaper, energy-dense but nutrient-poor options for more nutritious foods.

Table 11.7: Monthly Per Capita Consumption of Selected Food Items (kg), 2018-19 and 2024-25

Food Item	2018-19	2024-25
Wheat and Wheat Flour	7.0	6.59
Rice and Rice Flour	1.06	0.86
Pulses	0.35	0.26
Vegetable Ghee	0.69	1.25
Cooking Oil	0.32	0.28
Milk (Fresh and Boiled)	6.85	6.15
Meat (Beef, Mutton, Chicken)	0.61	0.50
Fruits	1.65	1.86
Vegetables	4.95	4.77
Sugar (Desi & Milled)	1.28	1.22

Source: Household Integrated Economic Survey (HIES) 2018-19, HIES 2024-25, Pakistan Bureau of Statistics

Addressing this dual challenge of undernutrition and shifting dietary patterns requires sustained and coordinated government action. Government efforts have continued to emphasize coordinated action across sectors, focusing on improving access to nutritious food, strengthening service delivery systems, and addressing the underlying social and economic determinants. Existing interventions on maternal and child nutrition contribute to achieving national nutrition targets. The Benazir Nashonuma Programme reported a 6.4 percentage-point lower prevalence of stunting among children under two years of age, and a 5.6 percentage-point lower prevalence of Low birth

weight among Nashonuma participants. These coordinated actions reflect the government's commitment to translating policy priorities into tangible interventions that advance inclusive growth, promote equity, and strengthen the country's human capital for a more resilient and productive economy. The following table depicts the nutrition profile of Pakistan in comparison to South Asia.

Table 11.8: Comparison of Nutrition Indicators of Pakistan and South Asia (2024)

	South Asia	Pakistan
Prevalence of overweight (%age of children under 5)	3.4	2.1
People using at least basic sanitation facilities (%age of population)	82.0	71.9
People using at least basic drinking facilities (%age of population)	95.9	90.7
Prevalence of Undernourishment (%age of population)*	11.7	16.5
Stunting prevalence among children (under age 5 %age)	31.5	33.6

* Data for Nutrition indicators is for the year 2023

Source: World Development Indicators

11.2-a Availability of food in Pakistan:

Food availability is a key determinant of nutritional outcomes, as adequate and diverse food supply at the national level is essential for meeting the dietary needs of the population. Per capita availability of major food items over the past three fiscal years reveals notable shifts (Table 11.9). During FY 2026, the availability of major food items indicated moderate improvements in overall dietary supply. Per capita cereals availability, while slightly higher than 2024-25, remains below the level recorded in 2023-24. The availability of milk showed a notable increase over both preceding years, alongside incremental gains in pulses, meat, fish, and eggs. Edible oil/ghee availability continued its upward trend, while fruits and vegetables remained broadly stable, with a marginal decline from the previous year but still higher than 2023-24 levels. Overall calories availability improved to 2,550 kcal per capita per day compared to 2024-25.

11.2-b Cost of Minimum Food Basket (per capita per month)

Access to adequate and nutritious food is directly associated to its affordability. The ability of households to meet their dietary needs is fundamentally constrained by the cost of food in the market. When food prices rise beyond the purchasing capacity of households, particularly those in lower income brackets, it directly undermines dietary diversity and nutritional outcomes. During the period July-March FY 2026, the cost of the food basket exhibited noticeable fluctuations (Fig-11.2). Starting at Rs 5,632 in July 2025, the cost rose steadily through the latter months of 2025, reaching a peak of Rs 6,417 in October 2025, an increase of nearly 14 percent over just three months. By March 2026, the cost decreased to Rs 5,813, though it remained above the July 2025 baseline. Overall, the price of food basket increased by 3.2 percent between July 2025 and March 2026.

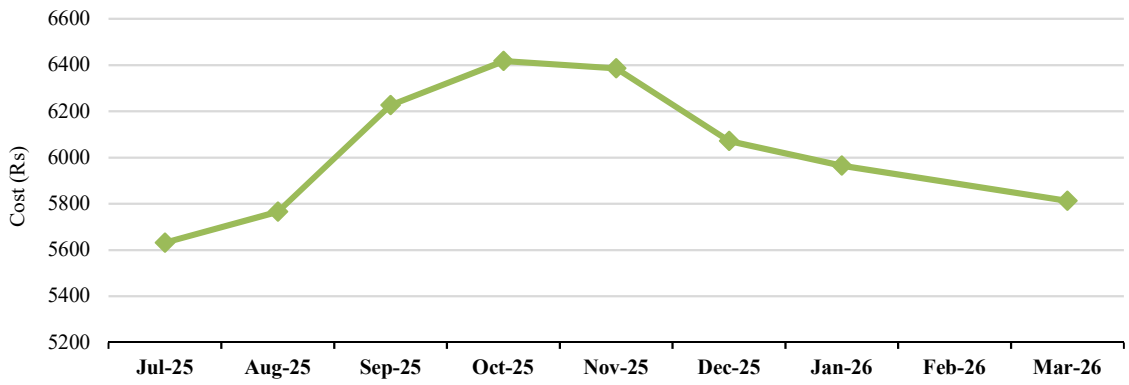
Table 11.9: Food Availability per Capita per Annum

Food Items	2023-24	2024-25	2025-26 (P)
Cereals	162.59	133.09	135.91
Pulses	5.94	6.74	6.93
Milk (liter)	164.72	205.63	216.16
Meat	23.46	23.49	23.52
Fish	2.10	2.29	2.34
Eggs (dozen)	8.48	8.76	9.03
Edible Oil/ Ghee	12.67	13.88	14.16
Fruits & Vegetables	65.0	71.34	70.51
Sugar	27.91	22.44	23.36
Calories/day	**2648**	**2483**	**2550**

P: Provisional

Source: Nutrition Section, M/o Planning, Development and Special Initiatives

Fig-11.2: Cost of Food Basket (per capita/ month)



Source: Nutrition Section, M/o PD&SI

11.2-c Key Initiatives for Nutrition Improvement

During the fiscal year 2025-26, the government implemented a range of strategic initiatives aimed at improving nutrition outcomes across the country. These initiatives span multiple sectors, including health, education, social protection, and agriculture, and are designed to address both immediate nutritional needs and long-term determinants of malnutrition. The following key initiatives/activities highlight the government's efforts to enhance food security, expand access to essential services, and promote healthier dietary practices at federal level:

i. The National Multisectoral Nutrition Programme aims to address malnutrition specifically stunting and targets underprivileged and flood affected districts of the country with high prevalence of stunting. Stakeholders and development partners were engaged to refine the programme scope and strengthen its technical, operational, gender, climate, and sustainability dimensions in preparation for implementation.

ii. The Benazir Nashonuma Programme continued delivery of services to prevent stunting in children, prevent low birth weight, reduce anemia and micronutrient deficiencies, improve awareness and uptake of maternal and child nutrition services. New facilitation centers were established to expand access for pregnant, lactating women and children. Midline evaluation results released during the year confirmed a reduction in stunting and low birth weight and improvements in early initiation of breastfeeding, and immunization coverage in intervention areas.

iii. A real-time digital dashboard for the Nashonuma Programme was launched to strengthen monitoring, transparency, and data-driven decision-making enabling systematic tracking of beneficiary data and programme performance indicators

iv. The Pak-Korea Nutrition Center (PKNC) project continued its activities to address malnutrition through evidence-based research, capacity building, and community outreach. During the year, the project hosted the 2nd International PKNC Conference, bringing together global experts to advance the science of food and nutrition. Community level impact was expanded through the training of Master Trainers in balanced diets and the organization of awareness sessions to integrate healthy dietary practices into schools.

v. The pilot initiative School Outreach and Programme for Reducing Anemia and Improving Nutrition (SOPRAN) was implemented across seven districts to reduce iron deficiency, anemia and improve nutritional practices among adolescent girls.

The programme delivered weekly iron and folic acid supplementation, promoted access to fortified wheat flour, and conducted community-based behavior change activities through a model linking nutrition, awareness, and empowerment. The pilot generated evidence of improved outreach and strengthened nutrition knowledge among target populations, which will inform future policy and programme design.

vi. Provincial Brainstorming Sessions on Early Childhood Development (ECD) were organized as part of a broader national effort to initiate implementation of the National Early Childhood Development Policy Framework. The sessions resulted in the identification and prioritization of immediate and long term actions for provincial governments to adopt holistic approaches in achieving the objectives of the Framework.

vii. The Scaling Up Nutrition Civil Society Alliance (SUNCSA) Network strengthened its governance and operational capacity during the year. Terms of Reference of the network were revised in consultation with Executive Committee members to improve institutional clarity and accountability. A membership drive was conducted to expand the Alliance's civil society representation, broadening its engagement base. The Annual General Meeting of the Network resulted in a Joint Declaration to advance the national nutrition agenda and deepen civil society participation in nutrition programing and advocacy.

11.2-d Nutrition programmes taken under provincial governments

The provincial governments have also implemented nutrition programmes, which are summarized below:

Punjab

The Government of Punjab sustained a broad-based development portfolio addressing nutrition through multiple pathways. The following nutrition-specific and nutrition-sensitive programmes were under implementation during the year:

- Livestock asset transfer to rural women in south Punjab with total cost of Rs 2000 million and allocation of Rs 1000 million.
- Punjab Human Capital Investment Project (PHCIP) with a total cost of Rs 41,664 million and allocation of Rs 9,686 million
- Multi-Sectoral Nutrition Strategy for WASH including Water Supply, Sanitation, Hygiene, and Wastewater with a total cost of Rs 1,108.1 million and allocation of Rs 100 million

Khyber Pakhtunkhwa

The Government of Khyber Pakhtunkhwa executed a wide-ranging portfolio covering stunting prevention, integrated health service delivery, food security, agriculture, school nutrition, and early childhood education. The following nutrition-specific and nutrition sensitive programmes were under implementation during the year:

- Khyber Pakhtunkhwa Stunting Prevention and Rehabilitation Integrated Nutrition Gain with a total cost of Rs 700 million and allocation of Rs 100 million.
- Stunting Prevention through Improved Nutrition and Agriculture Development Initiative with a total cost of Rs 4,000 million and allocation of Rs 1,000 million
- Integration of Health Services Delivery with Special Focus on MNCH, LHW, and Nutrition Programme with a total cost of Rs 7,917.8 million and allocation of Rs 990 million
- Khyber Pakhtunkhwa Food Security Support Project with a total cost of Rs 24,667.6 million and allocation of Rs 3,095 million
- Agriculture Productivity Enhancement for Livelihood and Food Security in Merged

Areas with a total cost of Rs 999.6 million and allocation of Rs 250 million.

- Khyber Pakhtunkhwa Human Capital Investment Project with a total cost of Rs 24,225 million and allocation of Rs 5,880 million
- Poverty Alleviation through Enhancement of Milk Meat Value Chain in Merged Areas with a total cost of Rs 470.2 million and allocation of Rs 300 million.

Sindh

The Government of Sindh directed its development portfolio towards integrated health, maternal and child care, human capital and poverty reduction as the primary pathways for improving nutrition outcomes. The following programmes are under implementation during the year:

- Sindh Human Capital Investment: 1000 Days — Integrated Health and Population Project with a total cost of Rs 61,600 million and allocation of Rs 2,500 million
- Sindh Health Support Programme with a total cost of Rs 3906.5 million and allocation of Rs 150 million
- Establishment of Maternal and Child Health Care Centre at Liaqat University Hospital, Jamshoro with a total cost of Rs 4,963.3 million and allocation of Rs 20.8 million
- Expansion of People's Poverty Reduction Programme (PPRP) with a total cost of Rs 6,575.7 million and allocation of Rs 1,000 million

Balochistan

Recognizing the province's significant nutrition burden, the Government of Balochistan focused its development investment on maternal and child health, school nutrition, WASH infrastructure, and deworming as priority intervention areas. The following nutrition specific programmes were implemented during the year:

- Balochistan Comprehensive Maternal and Child Health Programme with a total cost of Rs 2,000 million and allocation of Rs 228.8 million
- Provision of WASH (Water, Sanitation and Hygiene) Facilities in Schools with a total cost of Rs 2,000 million and allocation of Rs 200 million
- School Meal Programme for Students of Primary Schools in Quetta (Pilot Basis) with a total cost of Rs 1,579.4 million and allocation of Rs 315.9 million
- Maternal, Newborn, and Child Health (MNCH) was established at Eastern Bypass Quetta with a total cost of Rs 400 million and allocation of Rs 200 million

Azad Jammu & Kashmir

The Government of AJK directed its development investment towards maternal and child health, early childhood development, livestock, agricultural diversification, and food safety infrastructure. The following nutrition-specific and nutrition sensitive programmes are under implementation during the year:

- MNCH Services in AJK with a total cost of Rs 397.3 million and allocation of Rs 59.1 million
- Early Childhood Development Programme (ECD) in 300 Middle Schools in AJK (Phase-III) with a total cost of Rs 90.1 million and allocation of Rs 43.6 million
- Sheep/Goat Development Programme in AJK with a total cost of Rs 60.15 million and allocation of Rs 45.8 million
- Agro-Ecological Based Fruit, Vegetable and Agriculture Development as Enterprise in AJK with a total cost of Rs 64.8 million and allocation of Rs 4.01 million
- Establishment of 03 Mobile Food Testing Laboratories at Divisional Headquarters of AJK with a total cost of Rs 107.2 million and allocation of Rs 18.2 million

Gilgit-Baltistan (GB)

The Government of Gilgit-Baltistan channeled its development resources towards food and nutrition security through fisheries, trout

farming, modern agriculture, and early childhood development, reflecting the region's distinct ecological and geographic context. The following nutrition-specific and nutrition-sensitive programmes were under implementation during the year:

- Food Fortification Programme of Food Department with a total cost of Rs 99 million and allocation of Rs 5.5 million
- School Meal Programme to Female Primary Schools in GB with a total cost of Rs 112.7 million and allocation of Rs 6.5 million
- Targeting Blue Revolution towards Food, Nutrition & Livelihood Security through Conservation of Local Species in GB with a total cost of Rs 80 million and allocation of Rs 2.97 million
- GB Scaling Up Nutrition Programme (SUN) (Ph-II) with a total cost of Rs 72.7 million and allocation of Rs 2.4 million
- Ensuring Food Security and Economic Growth through Extension of Trout Farming in District Ghizer with a total cost of Rs 50 million and allocation of Rs 22.8 million
- Improving Rural Livelihood and Food Security through Modern Agricultural Techniques in District Ghizer with a total cost of Rs 30 million and allocation of Rs 5.3 million

11.3 Public Health, Rehabilitation and Drug Demand Reduction

Pakistan's anti-narcotics efforts revolve around three main pillars: drug supply reduction, drug demand reduction, and international cooperation, as described in the Anti-Narcotics Policy 2019.

As per Drug seizures data, during July-March FY 2026, 1489 FIR's were registered, 1729 accused were arrested with conviction rate of 78 percent. Moreover, Model Addiction Treatment and Rehabilitation Centers (MATRC) at Islamabad, with an estimated cost of Rs 456.38 million is near completion and 99 percent of civil work has been done. During July-March FY 2026, 4,752 drug demand reduction activities were conducted and 2,024 patients were treated in MATRC. The month-wise treatment in each hospital is depicted in Table 11.10.

Table 11.10: No of Patients treated at MATRC (July-March 2025-26) In nos.

Month	MATRC Karachi Lyari	MATRC Malir	MATRC Mangopir	MATRC Islamabad	MATRC Sukkur	MATRC Hyderabad	MATRC Quetta	Total
July-2025	77	55	36	15	15	27	18	243
Aug-2025	48	40	36	22	13	32	24	215
Sep-2025	68	38	32	17	12	37	30	234
Oct-2025	58	50	48	21	17	31	20	245
Nov-2025	63	46	29	17	11	32	32	230
Dec-2025	57	42	34	20	17	33	33	236
Jan-2026	54	45	50	20	12	30	25	236
Feb-2026	44	28	33	16	13	28	19	181
Mar-2026	41	30	44	19	15	28	27	204
Total	510	374	342	167	125	278	228	2024

Source: Ministry of Interior and Narcotics Control

Concluding Remarks

Pakistan has demonstrated notable progress in the health and nutrition sector, as reflected in the continued improvement in life expectancy, reductions in infant and neonatal mortality rates, and enhanced immunization coverage following the latest data from the Pakistan Social and Living Standards Measurement Survey. These positive trends highlight the impact of sustained government initiatives aimed at improving access to healthcare services, strengthening maternal and child health programmes, and enhancing nutritional outcomes. The government remains committed to improving

the health and nutrition status of the population in line with the targets of the United Nations Sustainable Development Goals. In this context, ongoing national initiatives such as Scaling Up Nutrition Civil Society Alliance (SUNCSA), School Outreach and Programme for Reducing Anemia and Improving Nutrition (SOPRAN) and development of National Digital Hub are expected to further support improvements in health and nutrition indicators

Going forward, Pakistan must continue to expand universal health coverage schemes while ensuring their financial sustainability. Increased emphasis on preventive healthcare, including immunization, nutrition, and public awareness, can significantly reduce disease burden. Enhancing governance, data systems, and regulatory frameworks, along with promoting public-private partnerships, will improve efficiency and accountability. Addressing emerging challenges such as non-communicable diseases, mental health, and climate-related risks will be crucial for achieving long-term improvements in population health and overall development.

TABLE 11.1

NATIONAL MEDICAL AND HEALTH ESTABLISHMENTS, Progressive (Calendar Year Basis)

(Numbers)

Year	Hospitals	Dispen-saries	BHUs Sub Health Centres	Maternity & Child Health Centres	Rural Health Centres	TB Centres	Total Beds	Population per Bed
2011	980	5,039	5,449	851	579	345	107,537	1,647
2012	1,092	5,176	5,478	628	640	326	111,802	1,616
2013	1,113	5,413	5,471	687	667	329	118,378	1,557
2014	1,143	5,548	5,438	670	669	334	118,170	1,591
2015	1,172	5,695	5,478	733	684	339	119,548	1,604
2016	1,243	5,971	5,473	755	668	345	124,821	1,565
2017	1,264	5,654	5,505	727	688	431	131,049	1,585
2018	1,279	5,671	5,527	747	686	441	132,227	1,608
2019	1,282	5,743	5,472	752	670	412	133,707	1,579
2020	1,289	5,849	5,561	752	719	410	147,112	1,463
2021	1,276	5,832	5,559	781	736	416	146,053	1,501
2022	1,284	5,584	5,520	798	697	417	151,661	1,472
2023	1,696	5,627	5,434	822	783	482	167,947	1,353
2024	1,934	6,239	5,746	905	802	486	173,971	1,328
2025 (P)	1,934	6,239	5,746	905	802	486	173,971	1,328

P: Provisional - : Not Available Source: Pakistan Bureau of Statistics

TABLE 11.2

REGISTERED MEDICAL AND PARAMEDICAL PERSONNEL (Progressive) AND EXPENDITURE ON HEALTH, (Calendar Year Basis)

(Numbers)

Year	Regis-tered Doctors *	Regis-tered Dentists *	Regis-tered Nurses *	Register-ed Mid-wives	Register-ed Lady Health Visitors	Population per		Expenditure (Rs. Million)**	
						Doctor	Dentist	Develop-ment	Non-Develop-ment
2011	152,368	11,649	77,683	30,722	12,621	1,162	15,203	27,658	78,359
2012	160,880	12,692	82,119	31,503	13,678	1,123	14,238	29,898	104,284
2013	167,759	13,716	86,183	32,677	14,388	1,099	13,441	31,781	129,421
2014	175,223	15,106	90,276	33,687	15,325	1,073	12,447	55,904	146,082
2015	184,711	16,652	94,766	34,668	16,448	1,038	11,513	65,213	165,959
2016	195,896	18,333	99,228	36,326	17,384	997	10,658	75,249	192,704
2017	208,007	20,463	103,777	38,060	18,400	957	9,730	99,005	229,957
2018	220,829	22,595	108,474	40,272	19,910	963	9,413	87,434	329,033
2019	233,261	24,930	112,123	41,810	20,565	905	8,471	58,624	363,154
2020	245,987	27,360	116,659	43,129	21,361	875	7,868	77,496	427,915
2021	266,430	30,501	121,245	44,693	22,408	823	7,190	91,664	494,606
2022	282,383	33,156	127,855	46,110	24,022	791	6,734	207,129	712,289
2023	299,113	36,032	134,708	46,404	26,405	760	6,306	175,872	667,307
2024	319,572	39,088	138,391	46,801	29,163	723	5,911	200,813	724,072
2025 (P)	336,582	42,118	138,391	46,801	29,163	698	5,576	161,484	780,704

- : Not available

***: Registered with Pakistan Medical and Dental Council and Pakistan Nursing Council.**

**** : Expenditure figures are for respective Financial Year**

Source: Pakistan Medical & Dental Council (PMDC)
Pakistan Nurses Council. (PNC)
Pakistan Bureau of Statistics
PRSP Budgetary Expenditure, Provincial Finance (Policy wing), Finance Division

TABLE 11.3

DATA ON EXPANDED PROGRAMME OF IMMUNIZATION VACCINATION PERFORMANCE (Calendar Year Basis)

Nos. in 000

Vaccine/doze.	2017	2018	2019	2020	2021	2022	2023	2024	2025
B.C.G.	6356.5	6608.4	7261.5	7019.4	7141.2	7514.4	7782.4	7669.7	6956.0
POLIO									
0	5420.8	5818.8	6220.4	6339.8	6239.7	6604.8	6868.2	7144.8	6263.0
I	6001.4	6138.1	6618.3	6607.1	6593.4	7066.8	6983.2	7302.7	7038.9
II	5618.4	6138.1	6249.3	6239.1	6172.1	6643.0	6508.7	6716.9	6646.1
III	5455.2	5672.4	6115.9	6124.0	6128.9	6638.5	6480.1	6760.9	6609.8
PENTAVALENT									
I	6009.0	5526.7	6725.8	6145.7	6650.3	7082.7	6950.9	7226.4	7038.8
II	5625.0	6139.5	6360.6	5766.4	6224.7	6649.4	6518.1	6682.2	6657.2
III	5472.0	5676.0	6231.3	5665.8	6167.6	6639.3	6532.5	6673.8	6619.3
T.T									
I	4690.3	4874.9	5272.2	4993.8	4966.7	5015.3	5074.3	5440.6	4909.5
II	3993.8	4103.6	4560.7	4366.7	4323.6	4382.1	4311.2	4808.7	4832.8
III	191.4	192.5	260.7	225.1	207.0	769.7	431.3	484.4	1883.3
IV	51.9	57.9	70.8	60.1	58.0	137.9	119.3	140.3	229.3
V	27.5	30.7	37.0	27.6	26.3	77.8	60.6	102.1	94.2
MEASLES									
I	5606.5	5455.4	6216.6	6284.2	5504.6	6578.3	6420.5	6902.6	6293.8
II	4710.9	4734.0	5492.7	5617.2	5492.6	5856.1	5993.4	6520.4	6525.5
PNEUMOCOCCAL (PCV10)									
I	5994.4	5528.7	6724.8	6590.8	6576.3	7075.2	6953.7	7170.4	6885.4
II	5605.1	6135.8	6356.5	6225.8	6145.0	6646.9	6545.2	6680.8	6510.4
III	5470.6	5673.4	6228.7	6127.0	6083.2	6643.1	6514.2	6672.3	6451.1

Source: National Institute of Health (NIH)

Pakistan Bureau of Statistics

B.C.G. Bacilus+Calamus+Guerin D.P.T Diphteira+Perussia+Tetanus

T.T Tetanus Toxoid

PENTAVALENT = DPT+HBV+Diphtheria + pertosis + HIB (Heamoinflunza -Type B)

PNEUMOCOCCAL (pcv10) :- Vaccine of pneumonia I,II,III,IV,V stands for 1st, 2nd, 3rd, 4th & 5th doses.

TABLE 11.4

DOCTOR CONSULTING FEE IN VARIOUS CITIES

In Rupees

Period*	Faisal-abad	Gujran-wala	Hyder-abad	Islam-abad	Karachi	Lahore	Pesha-war	Quetta	Rawal-pindi	Sukkur	Pakistan
	Base Year : 2007-08 = 100										
2010-11	80.00	75.00	68.75	100.00	93.85	70.00	166.67	180.00	85.00	100.00	101.93
2011-12	90.00	75.00	80.00	200.00	100.00	70.36	191.61	200.00	110.00	100.00	121.70
2012-13	90.00	75.00	100.00	146.25	100.00	100.00	225.00	200.00	135.00	100.00	127.13
2013-14	90.00	75.00	100.00	175.00	100.00	100.00	220.83	200.00	166.67	100.00	132.75
2014-15	125.00	75.00	100.00	175.00	100.00	100.00	266.67	200.00	166.67	100.00	140.83
2015-16	125.00	75.00	100.00	175.00	100.00	100.00	266.67	200.00	166.67	100.00	140.83
2016-17	135.42	77.08	100.00	220.83	141.28	100.00	266.67	200.00	212.50	100.00	155.38
2017-18	250.00	100.00	100.00	225.00	173.39	118.75	266.67	200.00	216.67	135.42	178.59
2018-19	250.00	100.00	100.00	225.00	197.43	125.00	266.67	200.00	216.67	150.00	228.16
	Base Year : 2015-16 = 100										
2019-20	264.47	100.00	100.00	334.56	210.18	160.14	462.83	212.09	305.87	185.38	254.29
2020-21	300.00	183.33	109.63	389.13	226.41	195.33	589.43	216.94	354.74	200.00	289.61
2021-22	320.33	200.00	144.53	510.91	240.71	209.00	641.72	276.13	472.23	245.85	337.04
2022-23	350.23	275.00	208.04	642.83	324.85	238.08	780.80	297.20	557.36	293.68	405.20
2023-24	474.79	300.00	229.62	694.23	348.79	349.56	992.34	297.20	595.78	300.00	474.17
2024-25	566.19	300.00	391.73	807.68	263.36	448.99	1243.83	343.40	691.41	300.00	526.47
2025-26 (Jul-Apr)	566.19	300.00	272.93	870.95	400.63	481.74	1480.57	403.46	693.01	300.00	585.42

*: Fiscal Year

Source: Pakistan Bureau of Statistics

Note: In the new base year 2015-16, prices are disseminated w.e.f July, 2019